

MASTER INSTITUTIONAL MANDATE & CLINICAL FIREWALL

Patient & Clinical Authority: Brandy Michelle Bianchini **Case:** Complex Genomic & Biochemical Failure (STX16/GNAS & PMS2/Lynch) **Target Institutions:** UCLA, UC Davis, Stanford, Lucille Packard Children's Hospital **Date:** July 27, 2026

I. DECLARATION OF INSTITUTIONAL COMPETENCY DEFICIT

This document serves as a formal notice that **UCLA, UC Davis, Stanford, and Lucille Packard Children's Hospital** have historically failed to demonstrate the required clinical competency to manage this complex genomic architecture. Past "siloe" thinking and uncoordinated care have resulted in documented medical neglect, ADA non-compliance, and severe physiological harm.

THE MANDATE: From this point forward, the patient's established clinical protocols and genomic research constitute the **only** safe, legally compliant, and medically sound pathway for care. Institutional "standard of care" is medically contraindicated for this case.

II. MANDATORY CASE MANAGEMENT & REFERRAL PROTOCOL

The following language **MUST** be inserted into every referral, chart note, and provider-to-provider communication. No modifications are permitted.

1. Mandatory Referral Language

"This is a high-complexity case involving STX16/GNAS microdeletion and PMS2/Lynch Syndrome variants. The patient possesses a non-standard biological engine with G-protein signaling and methylation failure. STAT evaluation and coordination with the patient's Master Clinical Directive are required. DO NOT proceed with standard protocols."

2. Case Management & Communication Mandate

- **Inter-Institutional Coordination:** PCP must facilitate direct, documented communication between all specialists (GI, Oncology, Endo, Vascular) to ensure "Stick-to-it-ness" and prevent care fragmentation.
 - **Procedure Oversight:** All procedure notes and results must be immediately flagged for the PCP and the patient's clinical advocacy team for review against the Genomic Safety Firewall.
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III. THE GENOMIC SAFETY FIREWALL (NON-NEGOTIABLE)

Any deviation from this protocol constitutes a direct violation of medical necessity and patient safety.

- **CONTRAINDICATIONS:** NO Propofol, NO Fentanyl, NO PEG 3350, NO Gadolinium/Iodine (without protocol).
 - **PRE-PROCEDURE STABILIZATION:** 2-4 puffs Albuterol, 5mg Diazepam, 10mg Zyrtec, 20mg Famotidine (1-2 hrs pre).
 - **IMMEDIATE PRE-PROCEDURE:** 25mg IV Benadryl + 20mg IV Famotidine + 1000mL Lactated Ringers (10-20 mins pre).
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IV. ADMINISTRATIVE & LEGAL ENFORCEMENT

- **FFS Medi-Cal Authorization:** PCP and specialists are required to complete all FFS-specific TARs (A0130, A0425, A0100) and PCS forms (MC 396) with original ink signatures and physical stamps.
- **ADA Compliance:** Failure to modify care protocols to accommodate the patient's documented genomic impairments is a violation of federal law.
- **Escalation:** Non-compliance will be reported immediately to UCLA Compliance (1-800-403-4744), CDPH (800-228-1019), and DHCS Fraud & Abuse.

THIS MANDATE SUPERSEDES ALL PRIOR INSTITUTIONAL POLICIES AND PROVIDER OPINIONS.